PREOPERATIVE DIAGNOSIS: Severe lower urinary tract symptoms (LUTS) secondary to benign prostatic hyperplasia (BPH) refractory to dual medical therapy (tamsulosin and dutasteride), with recurrent episodes of acute urinary retention.

POSTOPERATIVE DIAGNOSIS: Benign prostatic hyperplasia with trilobar obstruction.

PROCEDURE PERFORMED: Transurethral resection of the prostate (TURP) utilizing continuous-flow bipolar resection system.

OPERATIVE TECHNIQUE: Under satisfactory spinal anesthesia, the patient was positioned in the dorsal lithotomy position with padded Allen stirrups. The external genitalia and perineum were prepped with betadine and draped sterilely. A 26-French Olympus continuous-flow bipolar resectoscope equipped with a 30-degree lens and continuous 0.9% normal saline irrigation was introduced into the urethra under direct video visualization.

Urethrocystoscopy demonstrated a normal anterior urethra. The prostatic urethra revealed severe lateral lobe impingement with kissing lateral lobes and a prominent obstructive median lobe projecting into the bladder neck. The urinary bladder demonstrated moderate trabeculation, multiple pseudodiverticula, and clear urine without calculi, mucosal tumors, or foreign bodies. Bilateral ureteral orifices were visualized in normal anatomical position on the interureteric ridge with active clear efflux.

Resection was initiated on the obstructive median lobe. Using a bipolar loop electrode with saline plasma vaporization and cutting settings (200W cut, 120W coag), deep gutters were created at the 5 and 7 o'clock positions down to the circular fibers of the bladder neck. The median lobe tissue was systematically resected down to the surgical capsule. Resection was then directed to the left lateral lobe, followed by the right lateral lobe from the 12 o'clock position down to the capsular boundary. The apical tissue adjacent to the verumontanum was carefully sculpted under direct visualization, taking rigorous precautions to preserve the verumontanum and external urinary sphincter.

Hemostasis was achieved throughout the case using roller-ball coagulation. Chips were evacuated from the bladder using an Ellik evacuator, yielding approximately 38 grams of prostatic tissue which was submitted for histopathologic examination. Final cystoscopy confirmed a wide, unobstructed prostatic channel with clear irrigant. A 22-French 3-way Foley catheter with a 30 mL balloon was inserted and placed on continuous bladder irrigation with clear return.